

Title: MEDICAL NECESSITY POLICY – Evaluation and Management (E/M) Services

Effective Date: 01/01/2024

Policy ID: CMS-MN-001

Summary:

This policy outlines the criteria under which Evaluation and Management (E/M) services are considered medically necessary under CMS guidelines.

1. CPT Code 99201 – Office or other outpatient visit for a new patient

- Medical Necessity: Covered if the patient presents with mild to moderate symptoms requiring a problem-focused history and exam.
- ICD-10 Supportive Diagnoses:
 - M54.5 – Low back pain
 - R10.9 – Abdominal pain, unspecified
 - J02.9 – Acute pharyngitis, unspecified
- Documentation Requirements:
 - Chief complaint
 - History of present illness
 - Limited physical exam
 - Medical decision making of straightforward complexity

2. CPT Code 11400 – Excision of benign skin lesion, trunk/arms/legs, diameter < 0.5 cm

- Medical Necessity: Covered only when the lesion is symptomatic (e.g., bleeding, painful) or if malignancy is suspected.
- ICD-10 Supportive Diagnoses:
 - D23.5 – Other benign neoplasm of skin of trunk
 - L72.3 – Sebaceous cyst
- Documentation Requirements:
 - Description of the lesion (size, location, appearance)
 - Reason for removal
 - Pathology report if available

3. CPT Code 99444 – Online evaluation and management service

- Medical Necessity: Covered for established patients for non-emergent conditions.
- ICD-10 Supportive Diagnoses:
 - R51 – Headache
 - Z00.00 – General adult medical exam without abnormal findings
- Documentation Requirements:
 - Patient communication record
 - Clinical rationale
 - Treatment plan summary

Exclusions:

- Services performed without corresponding ICD-10 justification will be denied.
- Claims missing progress notes or documentation will be flagged for audit.

Compliance Risk:

- Submitting E/M codes without matching documentation is considered a High Risk audit trigger.